

1CB-0-3

Carcinoma, mucinous 8480/3

Site: breast, NOS C50.9 1/28/11

Pathology Form

Specimen Information

Collected by: _____ Date: _____ Time: _____
 Preserved by: _____ Date: _____ Time: _____

confirm laterality -
 CQCF = (L) - need documentation
 CQCF should be marked
 as mucinous, *
 (Run thru back Path.) not IAC.

Qualified
 11-1-10

SPECIMEN TYPE (# of samples provided)							
Frozen		Paraffin Block		Blood/Serum/Plasma		Slide	
Diseased	Normal	Diseased	Normal	Diseased	Normal	Diseased	Normal
X	X	X	X			X	X
Time to LN2		Time to Formalin		Time to LN2			
10 min		10 min		12 min			

PATHOLOGICAL DESCRIPTION			
Primary Tumor			
Organ	Size	Extension of Tumor	Distance to NAT
Breast	2 x x cm	NA	2 cm
Lymph Nodes			
Location	# Examined	# Metastasized	
Axilla	6	3	
Distant Metastasis			
Organ	Detailed Location	Size	
NO			
Pathological Staging			
pT ₂ N ₁ M ₀		Stage: II B	
Notes:			
UUID: B3FCE052-931C-4798-A188-F04A2178A5F6 TCGA-C8-A12X-01A-PR Redacted			



Criteria	Yes	No
Diagnosis Discrepancy		X
Primary Tumor Site Discrepancy		X
HIPAA Discrepancy		X
Prior Malignancy History		X
Dual/Synchronous Primary Noted		X
Case (circle): QUALIFIED		DISQUALIFIED
Reviewed (initials):		

MUCINOUS

Microscopic Description

Histological Pattern											
Cell Distribution			+	-	Structural Pattern			+	-		
Diffuse					Streaming						
Mosaic					Storiform						
Necrosis					Fibrosis						
Lymphocytic Infiltration					Palisading						
Vascular Invasion					Cystic Degeneration						
Clusterized			+		Bleeding						
Alveolar Formation					Myxoid Change						
Indian File					Psammoma/Calcification						

Cellular Differentiation											
Squamous	+	-	Adenomatous	+	-	Sarcomatous	+	-	Lymphomatous	+	-
Squamoid Cell			Glandular cell	+		Round Cell			Large Cell		
Spindle Cell			Cell Stratification			Fibroblast			Small Cell		
Keratin			Secretion	+		Osteoblast			RS Cell/RS Like		
Desmosome			Intracyt. Vacuole			Lipoblast			Inflam. Cell		
Pearl			Gland formation	+		Myoblast			Plasma Cell		

Cellular Differentiation: Well ~~X~~ Moderate Poor

Nuclear Appearance				
Nuclear Atypia:	0	I	II	III
Aniso Nucleosis			+	
Hyperchromatism			+	
Nucleolar Prominent		+		
Multinucleated Giant Cell		+		
Mitotic Activity		+		

Nuclear Grade: +

IHC Data			
Marker	Result	Value	Date
ER	☐ Negative ☐ Positive		
PR	☐ Negative ☐ Positive		
Her-2/neu	☐ Negative ☐ Positive		
B-Cell Marker	☐ Negative ☐ Positive		
T-Cell Marker	☐ Negative ☐ Positive		
Other:	☐ Negative ☐ Positive		
Other:	☐ Negative ☐ Positive		

Final Pathology Report

Histological Diagnosis: Mucinous adenocarcinoma **Grade:** II

Comments:

Principal Investigator

Pathologist

Date

CONSOLIDATED DIAGNOSTIC PATHOLOGY FORM*

Microscopic Appearance:

1. Histological pattern:

CELL DISTRIBUTION				STRUCTURAL PATTERN			
	+	-			+	-	
Diffuse				Streaming			
Mosaic				Storiform			
Necrosis				Fibrosis			
Lymphocytic Infiltration				Palisading			
Vascular Invasion				Cystic Degeneration			
Clusterized				Bleeding			
Alveolar Formation				Myxoid Change			
Indian File				Psammoma/Calcification			

2. Cellular features:

Squamous	+	-	Adenomatous	+	-	Sarcomatous	+	-	Lymphomatous	+	-
Squamous Cell			Glandular cell			Round Cell			Large Cell		
Spindle Cell			Cell Stratification			Fibroblast			Small Cell		
Keratin			Secretion			Osteoblast			RS Cell/RS Like		
Desmosome			Intracyt. Vacuole			Lipoblast			Inflam. Cell		
Pearl			Gland formation			Myoblast			Plasma Cell		

Otherwise Specified:

D1: 60%, D2: 60%

2. Cellular Differentiation:

Well	Moderately	Poor
☒		

3. Nuclear Atypia:

Nuclear Appearance	0	I	II	III
Aniso Nucleosis		☒		
Hyperchromatism		☒		
Nucleolar Prominent		☒		
Multinucleated Giant Cell		☒		
Mitotic Activity		☒		
Nuclear Grade		☒		

Histological Diagnosis: *Infiltrating ductal carcinoma, mucinous type, G+T*

Comments:

*U, Mx: carcinoma metastasized to LN CQCF
Should state "mucinous"
2/3/11
for*

Date

Director, Research Pathology

PATHOLOGIST STAFF FOR RESEARCH USE ONLY).

*(INTEGRATED REPORT OF FINDINGS BY CONTRIBUTOR AND